

	EMERGENCY MEDICAL SERVICES AGENCY	POLICY NUMBER	710
	A Division of the Merced County Department of Public Health	Effective Date	11/2025
Authority: Health and Safety Code, Division 2.5, and California Code of Regulations, Title 22, Division 9, Chapter 4, 1797.220		Initial Date:	07/2022
		Review Date:	10/2028

RETURN OF SPONTANEOUS CIRCULATION (ROSC)	
ADULT	PEDIATRIC
Any patient who achieves ROSC needs time to stabilize for approximately 5 minutes, as most patients will re-arrest during that time frame. Take time to prepare for transportation, assess the patient, and perform a 12-lead ECG for proper patient destination.	Do not delay transport. Transport when immediately able and safe to do so.
STEMI Center Destination (Adult patient meeting at least one (1) of criteria 1 or 2; and meets criteria 3): 12-Lead ECG reads STEMI or Acute MI (or equivalent) - Patient defibrillated at any time - Systolic BP is 80 mmHg or greater	
BLS	
Assess Vitals / Obtain SpO ₂ / Oxygen. Titrate to SpO ₂ of 94% or higher / Secure Airway - Assist Ventilations, if needed. Blood Sugar Check – For Hypoglycemia refer to EMS Policy #724 - Altered Level of Conscious (ALOC) – Syncope. Temperature – Sustain normothermia. If patient loses ROSC, begin CPR and treat the patient per EMS Policy #702 - Cardiac Arrest.	
ALS	
Follow BLS procedures if applicable. Obtain 4-Lead ECG and 12-Lead ECG (12-Lead Adult Only) Vascular Access - IV/IO rate as appropriate Fluid bolus due to hypotension – Refer to EMS Policy #738 – Non-Traumatic Shock Capnography – Attempt to maintain EtCO ₂ 35 – 45 mmHg.	
<u>If patient condition does not improve with previous treatment and SBP remains hypotensive:</u> Push Dose Epinephrine - 10 mcg (1ml) slow IV/IO push Repeat every 1-5 minutes until hypotension is resolved.	

Signatures on File

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